

DISCHARGE SUMMARY - SYNTHETIC PATIENT DATA (NOT REAL)

Patient: Linda Martinez

Date of Birth: 09/12/1972

Discharge Date: 2026-06-14

Admit Date: 2026-06-11

Hospital: Riverside Community Hospital

Attending Physician: Dr. Thomas Lee

Primary Specialty: Endocrinology

DIAGNOSIS

Primary Diagnosis: Diabetic Ketoacidosis (DKA) - Type 2 Diabetes Mellitus

ICD-10 Code: E11.10

MEDICATIONS AT DISCHARGE

1. Insulin Glargine (Lantus) 20 units - Once daily at bedtime, subcutaneous injection
2. Insulin Lispro (Humalog) - Sliding scale with meals, see attached chart
3. Metformin 1000mg - Twice daily with meals
4. Lisinopril 10mg - Once daily for kidney protection

FOLLOW-UP APPOINTMENTS

- Endocrinologist Dr. Lee - within 3 days post-discharge
- Diabetes education class - scheduled for 1 week post-discharge
- Ophthalmologist - annual diabetic eye exam due

ACTIVITY RESTRICTIONS

- Light walking 20-30 minutes daily as tolerated
- Monitor blood glucose before and after exercise
- Avoid exercise if blood glucose above 300 mg/dL or below 80 mg/dL

DIETARY RESTRICTIONS

- Diabetic diet: limit carbohydrates to 45-60g per meal
- Avoid sugary beverages, juices, and high-sugar foods
- Do not skip meals - especially when taking insulin
- Consistent meal schedule to help stabilize blood sugar
- Stay hydrated with water or sugar-free drinks

GO TO THE EMERGENCY ROOM IMMEDIATELY IF YOU EXPERIENCE

- Blood glucose above 400 mg/dL and feeling unwell
- Blood glucose below 50 mg/dL and unable to eat or drink
- Fruity breath, severe nausea, vomiting, or abdominal pain
- Confusion, difficulty waking, or loss of consciousness

CALL YOUR DOCTOR WITHIN 24 HOURS IF YOU NOTICE

- Blood glucose consistently above 250 mg/dL for 2 days

- Blood glucose below 70 mg/dL (low blood sugar)
- Inability to eat or drink due to nausea
- Signs of infection: fever, sores on feet that are slow to heal
- Running low on insulin or supplies